MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 18 OF 28

MFT 718

Intimacy and Relationship Dynamics

15 teaching weeks · 1 practical · 1 worked case · 1 figure

SEMESTER THREE

1.71 units · Prerequisite: MFT 710

> “Intimacy is not a single thing, and sex is not the whole of it.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 718 — Intimacy and Relationship Dynamics

MSc Marriage and Family Therapy · Semester Three · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 710

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	**Intimacy and Relationship Dynamics**	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Intimacy is not a single thing, and sex is not the whole of it. This course integrates attachment, desire, communication, culture and the body — and gives you language for the conversations most couples cannot start.

How to use this book

The material is intimate by design. You will be asked to examine your own beliefs about sex, bodies, gender and obligation, because those beliefs will shape what you notice and what you avoid. Supervision is essential for this course; several exercises should be processed there.

1. Course description

The course covers the psychology and physiology of intimacy and desire, the major sexual concerns presenting in couple therapy, the relational dynamics of pursuit and distance, betrayal and repair, the effects of culture and religion, and the practical skills of talking about sex.

It is explicitly **not** a sex therapy certification. It is the level of competence a family therapist needs to assess sexual concerns confidently, intervene relationally, and refer appropriately.

2. Learning outcomes

#	Outcome	Assessed
LO1	Assess intimacy across multiple domains	Observed assessment
LO2	Explain desire, arousal and response, including discrepancy	Exam
LO3	Conduct a respectful sexual history	Skills test
LO4	Formulate sexual concerns relationally and identify medical rule-outs	Case study
LO5	Deliver sensate focus and communication interventions	Skills test
LO6	Address culture, religion and consent	Reflective portfolio
LO7	Recognise the limits of your competence and refer	Viva

3. Assessment

Component	Weight	Due
Sexual history observed (30 min)	30%	Week 6
Case formulation and intervention plan (2,500 words)	40%	Week 11
Examination	20%	Week 16

Component	Weight	Due
Reflective portfolio	10%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	What intimacy is: eight domains	Domain assessment
2	Attachment and intimacy	Attachment-informed formulation
3	Desire: responsive and spontaneous	Desire mapping
4	The sexual response and its variation	Physiology teaching practice
5	Taking a sexual history	Observed practice
6	**Sexual history assessed**; talking about sex with couples	Language lab
7	Desire discrepancy and negotiation	Negotiation rehearsal
8	Common concerns I: erectile and arousal difficulties	Biopsychosocial formulation
9	Common concerns II: pain, orgasm, low desire	Referral decision-making
10	Betrayal, infidelity and the repair of trust	Disclosure planning
11	**Case plan due**; culture, religion and sexual scripts	Cultural genogram of sex
12	Consent, coercion and obligation	Boundary and consent work
13	Sexuality across the lifespan and in special populations	Adaptation planning
14	Sensate focus and intimacy exercises	Exercise design
15	Integration: the therapist's own comfort	Viva

Week 1 — What intimacy is

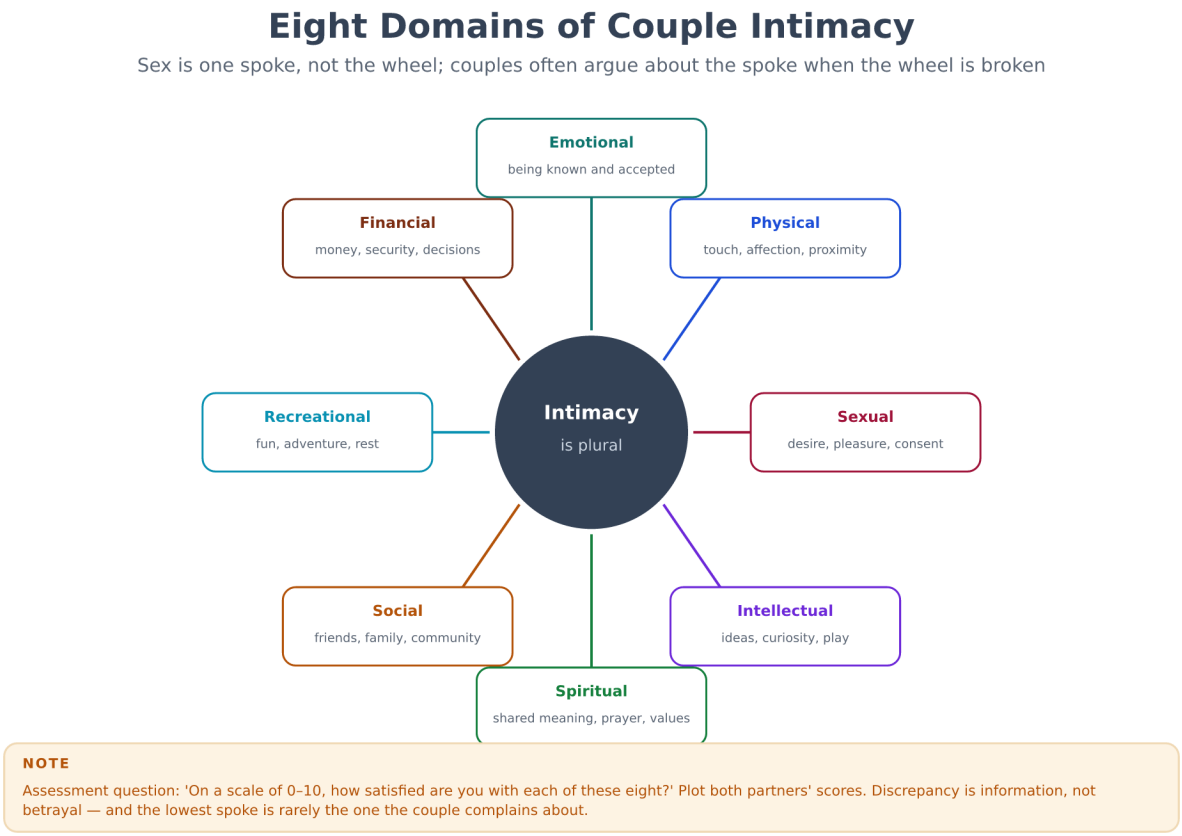


Figure 1.1 — Eight domains of couple intimacy

Couples rarely complain about "intimacy". They complain about sex, or loneliness, or a partner who is always on the phone. Your task is to locate the complaint in the wider system of intimacy domains: emotional, physical, sexual, intellectual, spiritual, social, recreational, financial.

The common pattern. A couple presents with a sexual problem, and assessment reveals that the emotional domain has been starved for years. Treating the sexual symptom alone usually fails; rebuilding the other domains usually helps the sexual one.

Assessment question. "On a scale of 0 to 10, how satisfied are you with each of these eight areas?" Plot both partners' answers. The gap between them, and the gap between the couple's stated problem and their lowest domain, is the clinical map.

Week 2 — Attachment and intimacy

Secure attachment allows both closeness and separateness: the person can ask for comfort and can tolerate a partner's independence.

Anxious attachment produces hypervigilance: sex may be used to secure reassurance, and any distance feels like rejection.

Avoidant attachment produces discomfort with dependency: sex may be easier than emotional closeness, and post-coital distance is common.

Disorganised attachment produces approach–avoidance: desire and fear in the same moment.

Clinical application. "What does sex mean to each of you?" often reveals the attachment function: proof of love, proof of adequacy, a way to avoid conversation, a way to end a conflict, a rare moment of safety.

Week 3 — Desire

Two models.

- **Spontaneous desire**: desire appears before arousal, seemingly unbidden. More commonly reported by men in many samples, and by people in new relationships.
- **Responsive desire**: desire emerges after arousal, in response to context, closeness, relaxation or stimulation. More commonly reported by women, by parents of young children, and by people under stress.

The clinical error. Treating responsive desire as "low desire" or "dysfunction". If a person's desire is responsive, the intervention is context — time, privacy, relaxation, non-sexual affection — not pressure to "want it more".

The dual control model (Bancroft & Janssen): sexual response depends on both excitation and inhibition. Stress, shame, performance anxiety, religious guilt and fear of pregnancy all raise inhibition. Lowering inhibition is often more effective than increasing stimulation.

Week 4 — The sexual response

The classic model (Masters & Johnson; Kaplan): desire → excitement → plateau → orgasm → resolution. **The circular model** (Basson) is more accurate for many people, especially women: intimacy, closeness, context and stimulation interact repeatedly, without a fixed sequence.

What clinicians need to know.

- Arousal is not desire, and lubrication is not consent or interest.
- Arousal non-concordance is normal: the body can show arousal without desire, and desire without arousal.
- Orgasm is not the goal; pleasure and connection are.
- Pain is always a medical question first (vaginismus, vulvodynia, endometriosis, infection, scarring, menopause).
- Medication (especially SSRIs) commonly affects desire, arousal and orgasm.
- Ageing changes speed and reliability, not the capacity for pleasure.

Week 5 — Taking a sexual history

Principles. Ask permission; explain why; use the person's language; be matter-of-fact; do not react with surprise, approval or disapproval; and take a history from each partner separately at least once.

A workable sequence.

1. Permission: "I ask everyone about this because it is often part of what brings couples here. Is that all right?"
2. Current: "How is intimacy between you at the moment? How often, and how satisfying, from 0 to 10?"
3. History: "Was there a time it was good? What was different then?"
4. Meaning: "What does sex mean to you — closeness, relief, duty, play, reassurance?"
5. Body: "Any pain, any difficulty with arousal or orgasm, any medical condition or medication?"
6. History of harm: "Has anyone ever pressured or forced you sexually?" (Ask individually.)
7. Culture and faith: "What were you taught about sex growing up? What do you believe is allowed?"
8. Expectations: "What would you like to be different — and what would your partner say you want?"

Language. Avoid euphemism and avoid clinical jargon. Use the words the couple uses, and offer a plain alternative if they have none.

> **ASSESSED**
> A 30-minute sexual history with a role-played client. Assessed on permission, pacing, breadth, non-judgemental tone, and the identification of medical or safety issues requiring referral.

Week 6 — Talking about sex

Why couples cannot. Shame, fear of hurting, fear of being judged, previous rejection, cultural prohibition, and the belief that "if you loved me you would know".

The therapist's job is to make the conversation normal, structured and safe. Techniques:

- **The third object.** Talk about "the sex thing" as an object between you, rather than about the people.
- **Scaling.** "How satisfied are you, 0–10?" Numbers are less exposing than adjectives.
- **Written disclosure.** Each writes three sentences about what they want more of; you read them aloud.
- **The permission sentence.** "I am going to ask something direct. You can say pass."

Scripts to give couples.

> "I want to talk about us, and I am nervous. Can we take twenty minutes on Saturday?" "I miss being close to you. I am not asking for sex tonight; I am asking whether we can talk about it this week." "When you turn away, I tell myself you do not want me. Is that what you mean?"

Week 7 — Desire discrepancy

The most common sexual complaint in couple therapy. It is a normal difference, not a disorder, until it produces distress.

The pattern. The higher-desire partner pursues; the lower-desire partner avoids; the pursuit increases pressure; the avoidance increases; both feel rejected, and the couple stops touching altogether.

Intervention.

1. **Normalise.** "Desire differences are the norm, not the exception. The problem is not the difference; it is the cycle around it."
2. **De-pressurise touch.** Reintroduce non-sexual affection with an explicit ban on sex for a period.
3. **Negotiate.** What is acceptable to both? Frequency is a poor target; quality, initiation and affection are better ones.
4. **Address the context.** Sleep, children, work, resentment, body image, medication.
5. **Reject the scoreboard.** Sex is not a debt; "I did it last time" is a symptom of the problem, not a solution.

Week 8 — Erectile and arousal difficulties

Always rule out medical causes: vascular disease, diabetes, hypertension, medication, hormonal factors, alcohol and smoking, neurological conditions. Refer to a physician; do not treat as purely psychological.

The psychological pattern. Performance anxiety → monitoring → arousal drops → confirmation of the fear → increased anxiety. The partner's disappointment (real or imagined) intensifies it.

Relational intervention. Reduce performance focus; reintroduce sensate focus; involve the partner in non-demanding intimacy; address the couple's broader pattern; and refer for medical assessment and, where appropriate, medication.

Arousal difficulties in women follow a similar logic, with a stronger contribution from context, pain, relationship quality and hormonal change.

Week 9 — Pain, orgasm and low desire

Presentation	First question	Relational work
Painful intercourse	Has a medical cause been excluded?	Pace, consent, fear, communication
Difficulty reaching orgasm	Is this new? Is it situational?	Reduce goal focus; explore what works alone
Absent or low desire	Is it global or partner-specific?	Context, resentment, responsive desire model
Aversion	Is there a history of coercion or trauma?	Safety, trauma-informed pacing, referral

Referral is not failure. Persistent pain, unexplained physical symptoms, severe trauma responses and psychiatric complexity belong with specialists. The relational work continues alongside.

Week 10 — Betrayal and repair

Betrayal trauma (developed in MFT 710). The betrayed partner's symptoms mimic PTSD: intrusion, hypervigilance, intrusive images, and a shattered assumption that the partner is safe.

Repair requires four things, in order.

1. **Safety** — the behaviour has stopped; boundaries are in place; no violence.
2. **Disclosure** — enough truth to end the lying, not so much detail that it becomes a weapon. Agree what will be asked and answered.
3. **Remorse that is not self-pity** — "I did this, it was my choice, and I see what it cost you."
4. **Consistency over time** — transparency, reliability, and patience with the betrayed partner's cycling.

The cycling is normal. Recovery is not linear: the betrayed partner may be calm for two weeks and then devastated by a song. Warn both partners, so the cycle does not become a second betrayal.

Sex after betrayal. Often the couple either avoids sex entirely or uses it to short-circuit the conversation. Both are understandable and neither is repair. Rebuild slowly, with explicit consent and no expectation.

Week 11 — Culture, religion and sexual scripts

Sexual scripts are culturally learned instructions about who should want what, when, and with whom.

Questions for the couple (each individually first).

- What were you taught about sex before marriage? By whom?
- What did you learn about your body?
- What is permitted, and what is forbidden? What happens if you break the rule?
- Is sex a duty, a gift, a right, a sin, a reward?
- Who initiates? Who is allowed to refuse? What happens when someone refuses?

Clinical themes in Nigerian and similar contexts.

- Pre-marital abstinence teaching that produces guilt after marriage.
- The expectation that sex is a marital obligation, which can silence refusal.
- Female sexual pleasure as unspoken or stigmatised.
- Bride price and the sense of "ownership" it can create.
- Family planning, fertility pressure, and the stigma of childlessness.
- Religious counselling that frames desire as sin.
- Polygyny, and the sexual dynamics between co-wives.

The clinical stance. You are not there to adjudicate the script; you are there to make it speakable, and to ensure that consent is real for both people.

> **CASE PLAN (40%)**
> A couple with a desire discrepancy and a history of religious guilt. Includes sexual history, relational formulation, medical rule-outs, intervention plan, and referral criteria.

Week 12 — Consent, coercion and obligation

Consent is ongoing, specific and freely given. It can be withdrawn at any point, including during sex, and including in marriage.

Coercion includes pressure, sulking, threats, guilt, economic withholding, and the use of religion. It does not have to be physical to be harmful.

Assess individually: "Can you say no to your partner without fear of what will happen? What does happen when you say no?"

When there is coercion, the work is with the person experiencing it first: safety, validation, and options. Couple work is contraindicated until safety is established.

Week 13 — Lifespan and special populations

Group	Considerations
Parents of young children	Exhaustion, touch saturation, interrupted privacy; plan rather than wait for spontaneity
Midlife	Hormonal change, body image, teenagers in the house, career strain

Group	Considerations
Later life	Assumption of asexuality by families and clinicians; health, medication, bereavement, new partners
Chronic illness and disability	Access, pain, fatigue, body image, and the partner's carer role
LGBTQ+ couples	Minority stress, disclosure, family rejection, and the absence of affirming services
Asexual people	Absence of sexual attraction is not pathology; negotiate intimacy on other terms
Post-cancer	Body change, fertility, fear, and the couple's avoidance

The clinician's assumption to avoid: that everyone wants sex, that everyone should, and that a relationship without sex is deficient. Ask what this couple wants.

Week 14 — Sensate focus

Purpose. To reduce performance anxiety and rebuild non-demanding physical intimacy.

Progression.

1. **Non-genital touch, no sex.** Twenty minutes, one partner touches, the other receives; then swap. Feedback only on what felt pleasant, not on what was missing.
2. **Genital touch, still no intercourse.** Continue the same structure.
3. **Intercourse permitted but not required.** Often the couple is advised to continue the exercises first.
4. **Generalisation.** Gradually return to the couple's own sexual repertoire.

Rules. No goal, no intercourse during the first stages, no negative feedback, and a clear opt-out at any time. Couples who "fail" at sensate focus are usually still performing.

Week 15 — The therapist's comfort

Your own sexual history, beliefs and discomfort will shape this work. Write 500 words on: what you find hardest to ask; what judgements you notice; and what you will do when you are out of your depth. Then defend it in a viva, and take it to supervision.

Glossary

Arousal non-concordance — mismatch between physiological arousal and subjective desire. **Desire discrepancy** — a difference between partners in the frequency or intensity of sexual desire. **Dual control model** — sexual response as the balance of excitation and inhibition. **Responsive desire** — desire that emerges in response to arousal or context. **Sensate focus** — graded, non-demanding touch exercises. **Sexual script** — culturally learned expectations about sexual behaviour and meaning. **Spontaneous desire** — desire experienced before arousal, without an obvious trigger.

Self-assessment

Multiple choice

1. Lubrication indicates: (a) consent (b) physiological arousal, which may not match desire (c) desire (d) orgasm

2. Responsive desire is best supported by: (a) pressure to initiate (b) context, relaxation and non-sexual affection (c) performance goals (d) frequency targets

3. The most common sexual complaint in couple therapy is: (a) infidelity (b) desire discrepancy (c) pain (d) infertility

4. Sensate focus requires: (a) intercourse from stage one (b) no goal and no intercourse initially (c) feedback on what is missing (d) daily practice

5. Erectile difficulties should first prompt: (a) psychodynamic exploration (b) medical rule-out (c) couple blame (d) medication advice

6. Consent in marriage is: (a) automatic (b) ongoing, specific and revocable (c) implied by the ceremony (d) not a clinical matter

7. Betrayal trauma symptoms resemble: (a) depression only (b) PTSD (c) psychosis (d) nothing

8. Asexual identity: (a) is a disorder (b) is not pathology; intimacy may be negotiated otherwise (c) requires treatment (d) means no relationships

9. Painful intercourse is first: (a) a relational problem (b) a medical question (c) a sign of avoidance (d) untreatable

10. The therapist should refer when: (a) the couple mentions sex (b) persistent pain, trauma complexity or issues beyond competence arise (c) the couple is young (d) culture differs

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Name the eight domains of intimacy and explain why the lowest is not always the complaint.
2. Distinguish spontaneous and responsive desire and give the clinical implication.
3. Write six questions from a sexual history.
4. What are the four stages of sensate focus?
5. How do you assess for coercion without leading the client?

Essay (choose one, 2,000 words)

1. "Desire discrepancy is a relationship problem, not a disorder." Discuss with a case.

2. Analyse the influence of religious teaching on sexual intimacy in couple therapy.
3. Discuss the ethical and clinical issues in working with sex in a couple where coercion is suspected.

Further reading

- Basson, R. (2001). Human sex response cycles. Journal of Sex and Marital Therapy, 27(1), 33–43.
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Next: MFT 719 — Family Life Cycle and Transitions